From first booking to the last review

A practical doctor workflow from morning to night, showing where Sahayak saves administrative effort and where the doctor stays in control.

What this guide explains

The patient can complete a standard appointment conversation without staff entering it manually. Once confirmed, the booking appears in Today. Check-in, queue and intake add context; the doctor sees a focused work list, conducts the visit, approves clinical output and follows outstanding care through one inbox.

Doctor’s complete day at a glance

Time	Doctor’s action	Sahayak support
Before clinic	Review Today, new bookings, urgent/overdue work and patients due for follow-up.	Shows bookings, queue, attention counts and recorded-data daily brief.
First arrival	Open the next patient, read intake and pre-visit brief.	Creates token/queue state and pulls source-linked history.
During visits	Call, consult, dictate/type, edit and sign.	Updates live queue; structures a source-preserving draft and tracks outstanding work.
After each visit	Approve instructions/plan, review checklist and finish.	Surfaces unsigned drafts, tasks, next appointment and check-in options.
At closing	Review inbox, unresolved replies and tomorrow’s schedule.	Keeps due and urgent items visible with owner and original source.

01 The appointment reaches the doctor

Step	What happens	Why it helps
1. Enquiry	Assistant asks visit type, required details/consent, doctor if needed, preferred time and then shows real slots.	Front desk can serve routine enquiries without interrupting a consultation.
2. Confirmation	Booking is saved only after explicit YES and slot recheck.	The doctor sees an actual confirmed appointment, not a tentative chat request.
3. Doctor Today	The appointment list shows time, patient, reason and status. The page also shows booked/seen/waiting counts.	The doctor can plan the session in one place. The app does not claim a push alert for every new booking.
4. Arrival	Patient self-checks in or reception helps; queue/token appears. Intake questions may be answered while waiting.	The doctor sees who is present and has context before calling them.
5. Call next	Doctor uses Next patient; queue and waiting display update.	Less verbal coordination between room and front desk.

What the doctor should check before opening the room

- Patient identity, booked reason and whether this is a first visit or follow-up.
- Intake answers and original wording, previous signed notes, uploaded documents, verified medicines/allergies and outstanding tasks.
- Urgent alerts or patient messages that should change the order or require immediate human action.

02 Consultation: keep the record useful

A repeatable visit sequence

Moment	Doctor action	Sahayak action
Start	Mark the called patient as present/in consultation.	Tracks the explicit consultation start used for duration measures.
Review	Open Before the consultation and inspect source links.	Shows dated concerns, intake, signed notes and verified facts without inventing a diagnosis.
Document	Dictate in the browser or type history, assessment, differential, investigations and instructions.	Keeps original text beside a structured editable draft. Missing sections remain empty.
Check	Compare reports/results to originals; review allergy and active-medicine prompts.	Flags exact text matches and outside-range recorded values; it does not perform comprehensive clinical interpretation.
Sign	Edit and sign the note and any prescription/certificate draft.	Records final doctor approval and preserves draft/signature state.

When the day does not go to plan

- If running late, enter the delay, review the affected-patient preview, then confirm before notifications or rebooking work proceeds.
- If a patient is absent, hold/skip or let reception mark a no-show; the queue remains visible to everyone.
- If an urgent patient message appears, open its source conversation, acknowledge it and direct a human response. Viewing it alone does not resolve it.

Doctor’s control point

Clinical text is never sent simply because it was drafted. The doctor signs notes and approves patient instructions, translations and care plans after checking the source.

03 After each visit: close the care loop

The five-minute finish

- **Review the checklist:** signed/unsigned note, draft prescriptions or certificates, next booking, instruction drafts, plan drafts, unverified entries and older open tasks.
- **Approve instructions:** copy from a signed plan, edit for the patient and approve. A translation remains a draft until separately reviewed.
- **Plan follow-up:** select approved template or patient-specific steps, owner and due time; approve the plan. Local follow-up contact is recorded manually today.
- **Schedule a check-in:** with agreement, consent and an owner, set a post-visit check-in. The completion screen can show a configured automatic scheduling preview; delivery still depends on an enabled live channel.
- **Finish:** acknowledge outstanding items have an owner, then close the visit.

When the patient replies later

The inbox brings back due work, unresolved patient messages, check-in concerns, report/referral tasks and urgent alerts. Staff can record contact, but the doctor reviews clinical concerns and signs off on clinical resolution. A patient who says they still need help reopens the concern; silence does not close it.

What this saves the doctor from doing repeatedly

Copying confirmed appointment details into another list; asking reception who is waiting; rebuilding the patient story from scattered records; remembering every unsigned draft or due follow-up; and searching several places for urgent items. Actual time saved should be measured during a clinic pilot.

04 End of day and next level of automation

Before leaving

- Review urgent and overdue inbox items, unsigned notes, unreviewed reports and patient replies still waiting for a decision.
- Check tomorrow’s appointments, doctor availability and any known schedule change with the office operator.
- Confirm every unresolved clinical task has a responsible person and due date. Defer only nonurgent work appropriately.
- Use clinic measures for recorded no-shows, follow-up completion, workload and consultation duration; do not treat these as proof of clinical outcomes or time savings.

Automation to add in a safe order

Stage	What changes	Doctor’s remaining action
1. Reliable messages	Connected channels deliver consented booking/reminder/check-in messages with receipts and failures.	Review exceptions and clinical replies.
2. Closed-loop follow-up	Approved plan steps trigger outreach; replies are matched to the right patient and task.	Approve the plan, review concerning replies and decide completion.
3. Richer preparation	Validated source extraction and draft agendas bring more context before the visit.	Verify facts and make the clinical judgment.
4. Advanced medicine checks	A maintained clinical knowledge source supports broader interaction/dose checks.	Review evidence and decide treatment; requires clinical validation before use.

The core promise

Sahayak can run the administrative path around a consultation so the doctor works from a prepared appointment, a live patient queue and a single follow-through list. The doctor remains responsible for care and clinical approval.

Source basis: apps/web/app/doctor/page.tsx, components/AttentionSummary.tsx, components/VisitChecklist.tsx; docs/CLINIC_APPOINTMENT_FLOW.md; docs/local-workflows.md; docs/care-workflows.md; docs/product-roadmap.md; docs/patient-checkins-plan.md.